SACTCHECK · NCCP 00464

My complete treatment handbook

Dacarbazine — metastatic melanoma

All information can be read on paper. Keep this with your treatment record.

> Use the urgent instructions and contact numbers in the following companion pages. Example telephone numbers must be replaced by your team before patient use. Your individual treatment plan takes precedence.

Contents

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2. Full treatment reference: medicines, schedule, tests, symptoms and follow-up — page 5
3. Detailed treatment and monitoring plan — page 24
4. Supportive medicines and care — page 27
5. Visits, access and practical arrangements — page 29
6. Your treatment passport and symptom record — page 30
7. Wallet card and phone instructions — page 34

SACTCheck v0.84.42 · Clinical, pharmacy and patient review pending. Formal consent is supplied separately.

Dacarbazine — metastatic melanoma

To control metastatic melanoma with chemotherapy, with benefit, blood counts and tolerability reviewed before each treatment.

Advanced melanoma · NCCP 00464 v3 · Document revised 6 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

This is single-agent dacarbazine at the NCCP 850 mg/m² dose. It is not an immune-checkpoint or BRAF-targeted regimen. Historical studies often used 1000 mg/m² and different treatment settings; their response or survival figures are not a precise forecast for this 850 mg/m² course.

My overall treatment

Dacarbazine by IV infusion on day 1, repeated every 21 days for up to six cycles, or stopped earlier for progression or unacceptable toxicity.

Treatment course
Day 1 every 21 days, up to 6 cycles; may stop earlier.

A typical 21-day cycle

Day 1	Day 2–21
Treatment day	Between treatment visits

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before	**Baseline safety** NCCP / current product information / individual plan	Blood count, kidneys/liver, medicines and fertility discussion.
During	**Safety checks** NCCP / current product information / individual plan	Blood count, kidneys/liver before every cycle; extra checks as needed. Lowest counts may occur 3–4 weeks after a dose.
After / response	**Follow-up** NCCP / current product information / individual plan	Blood recovery, symptom review and response scans individually planned.

Between treatments

- Infusion one hour plus checks and anti-sickness treatment. Call for fever/unwellness, bleeding, persistent vomiting or severe abdominal pain; report infusion-site burning immediately.

Bring medicine list, test dates and actual oncology/out-of-hours numbers. Example numbers must be replaced.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Energy / concentration

Dacarbazine

Tiredness, anaemia, poor sleep and nausea can affect concentration. Report severe dizziness, confusion or a major change in function; do not drive if unwell.

3 Breathlessness / anaemia

Dacarbazine

Low red cells can cause tiredness or breathlessness. New or worsening breathlessness needs assessment; severe breathlessness or chest pain is an emergency.

5 Liver

Dacarbazine

Rare severe liver injury can cause abdominal pain, fever and jaundice. Seek urgent assessment; routine liver tests do not replace symptom reporting.

7 Stomach / appetite

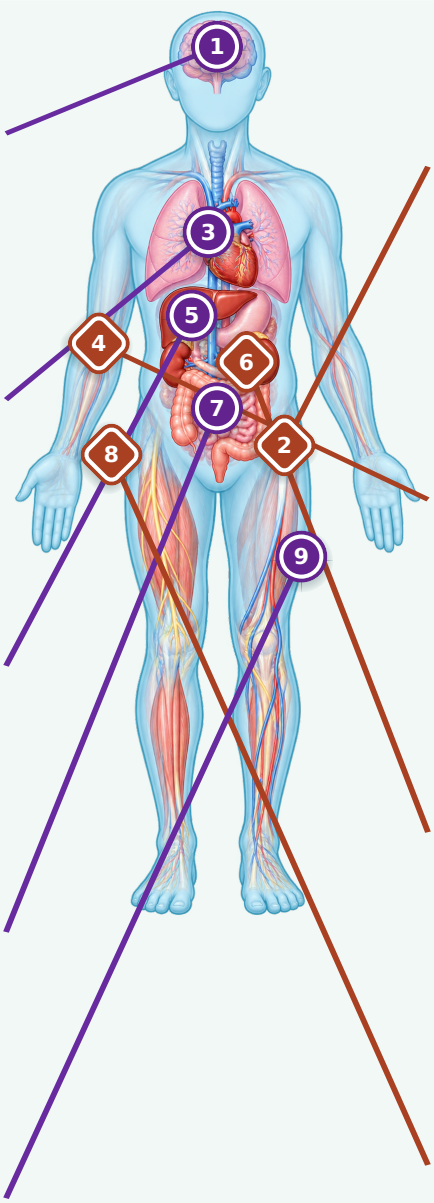
Dacarbazine

Nausea and vomiting need preventive medicines and a take-home plan. Report persistent symptoms, severe abdominal pain or failure of rescue treatment.

9 Bruising / bleeding

Dacarbazine

Low platelets can cause bruising, nosebleeds or bleeding gums. Persistent bleeding, black stools or blood in urine require prompt assessment.



2 Blood / infection

Dacarbazine

Low white cells increase infection risk and may occur late in the cycle. Fever or suddenly feeling unwell needs urgent contact, even near the next planned dose.

4 Skin / hair

Dacarbazine

Hair loss or sun sensitivity can occur. New severe rash, swelling or breathing difficulty may indicate allergy and needs urgent help.

6 Kidneys / fluids

Dacarbazine

Vomiting and reduced intake can lead to dehydration. Report reduced urine, persistent vomiting or inability to drink; kidney tests also help guide dosing.

8 Infusion site

Dacarbazine

Pain, burning, swelling or redness during infusion can mean leakage outside the vein. Tell the nurse immediately because tissue injury is possible.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Follow your unit’s fever threshold and urgent-contact instructions; call immediately if you develop fever, shaking chills or feel suddenly unwell, even without a high temperature. Also report unusual bleeding, persistent vomiting, inability to drink, reduced urine, jaundice or severe abdominal pain. Fever with abdominal pain and jaundice needs urgent assessment for rare severe liver injury. Tell the nurse immediately about infusion-site burning or swelling.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital’s temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Keep your treatment details to hand

Advanced melanoma · NCCP 00464 v3 · Document revised 6 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Dacarbazine

Dacarbazine by IV infusion on day 1, repeated every 21 days for up to six cycles, or stopped earlier for progression or unacceptable toxicity.

Phase: _____ Planned cycles: 6

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Team stamps after each completed cycle.

C1	C2	C3
Date / initials	Date / initials	Date / initials
C4	C5	C6
Date / initials	Date / initials	Date / initials

My treatment information

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

NCCP 00464 · VERSION 3

Your treatment, explained

Advanced melanoma · NCCP 00464 v3 · Document revised 6 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Dacarbazine — metastatic melanoma

Reference treatment cycle

Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Follow your unit's fever threshold and urgent-contact instructions; call immediately if you develop fever, shaking chills or feel suddenly unwell, even without a high temperature. Also report unusual bleeding, persistent vomiting, inability to drink, reduced urine, jaundice or severe abdominal pain. Fever with abdominal pain and jaundice needs urgent assessment for rare severe liver injury. Tell the nurse immediately about infusion-site burning or swelling.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your treatment and urgent-contact card is supplied separately and in the wallet-card section of the complete handbook.

Sources: nccp · dacarbazine-pi

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

How each drug works
Your treatment phases
Your 21-day timeline
Baseline and cycle-specific checks
Whole treatment, scans and checks
Tablets, visits and practical planning
Understanding your diagnosis and treatment goal
Your medicines and overall plan
Taking or receiving treatment safely
Tests, results and treatment decisions
What to expect between visits
Skin, body changes and daily care
Fertility, pregnancy and breastfeeding
Decisions, support and useful records
After a treatment hold, change or completion
Supportive medicines
Understanding the evidence
Questions to ask
Sources

UNDERSTAND YOUR TREATMENT

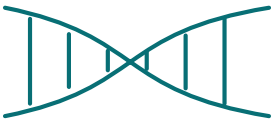
What each medicine does

To control metastatic melanoma with chemotherapy, with benefit, blood counts and tolerability reviewed before each treatment.

CYTOTOXIC CHEMOTHERAPY

Dacarbazine

DTIC



Damages cancer-cell DNA

Dacarbazine is converted in the body into substances that damage DNA and make it harder for cancer cells to divide. It also affects some healthy cells, including blood-forming cells. This explains why nausea prevention and blood-count checks are central to the plan.

Damages cancer-cell DNA → Treatment plan and safety checks → Report new symptoms early

What to watch for

Report fever or feeling acutely unwell, bleeding, persistent vomiting, severe abdominal pain or burning at the infusion site.

Sources: nccp · dacarbazine-pi

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The anatomical side-effect map follows.

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Energy / concentration

Dacarbazine

Tiredness, anaemia, poor sleep and nausea can affect concentration. Report severe dizziness, confusion or a major change in function; do not

3 Breathlessness / anaemia

Dacarbazine

Low red cells can cause tiredness or breathlessness. New or worsening breathlessness needs assessment; severe breathlessness or chest pain

5 Liver

Dacarbazine

Rare severe liver injury can cause abdominal pain, fever and jaundice. Seek urgent assessment; routine liver tests do not replace symptom

7 Stomach / appetite

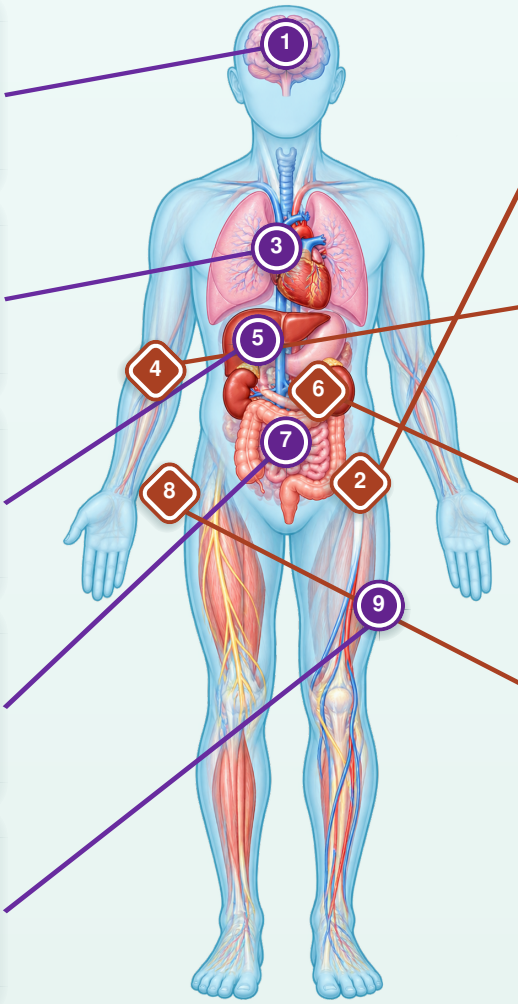
Dacarbazine

Nausea and vomiting need preventive medicines and a take-home plan. Report persistent symptoms, severe abdominal pain or

9 Bruising / bleeding

Dacarbazine

Low platelets can cause bruising, nosebleeds or bleeding gums. Persistent bleeding, black stools or blood in urine require prompt assessment.



2 Blood / infection

Dacarbazine

Low white cells increase infection risk and may occur late in the cycle. Fever or suddenly feeling unwell needs urgent contact, even near the

4 Skin / hair

Dacarbazine

Hair loss or sun sensitivity can occur. New severe rash, swelling or breathing difficulty may indicate allergy and needs urgent help.

6 Kidneys / fluids

Dacarbazine

Vomiting and reduced intake can lead to dehydration. Report reduced urine, persistent vomiting or inability to drink; kidney tests also help guide

8 Infusion site

Dacarbazine

Pain, burning, swelling or redness during infusion can mean leakage outside the vein. Tell the nurse immediately because tissue injury is possible.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

Your treatment phases

21-DAY REFERENCE CYCLE

Treatment visits

Treatment visits | **Repeat only as prescribed**

Dacarbazine by IV infusion on day 1, repeated every 21 days for up to six cycles, or stopped earlier for progression or unacceptable toxicity.

Dacarbazine by IV infusion on day 1, repeated every 21 days for up to six cycles, or stopped earlier for progression or unacceptable toxicity.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Your testing and review plan

A reference cycle is not a personalised calendar. Tests listed as symptom-triggered or individual are not presented as mandatory tests at every visit. No universal imaging or tumour-marker interval is invented.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00464 v3: current regimen source · HPRA Dacarbazine 200 mg product information, March 2024

THE WHOLE TREATMENT JOURNEY

Dacarbazine — metastatic melanoma: treatment and checks

Advanced melanoma

Dacarbazine by IV infusion on day 1, repeated every 21 days for up to six cycles, or stopped earlier for progression or unacceptable toxicity.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms NCCP 00464	Full blood count, kidney and liver function, medicines and prior treatment history. Discuss pregnancy prevention and fertility before the first dose. Record symptoms and the disease sites being followed on scans.	Full blood count, kidney and liver function before every cycle. Blood-count suppression can be delayed: the product information describes the lowest counts often three to four weeks after a dose, so a 21-day appointment does not guarantee recovery. Extra blood tests are arranged when clinically needed.	Follow your unit's fever threshold and urgent-contact instructions; call immediately if you develop fever, shaking chills or feel suddenly unwell, even without a high temperature. Also report unusual bleeding, persistent vomiting, inability to drink, reduced urine, jaundice or severe abdominal pain. Fever with abdominal pain and jaundice needs urgent assessment for rare severe liver injury. Tell the nurse immediately about infusion-site burning or swelling.	Keep reporting fever, bleeding, worsening fatigue or other symptoms after the last infusion while counts and organs recover. Response scans, later blood tests and the next treatment decision are individual; the NCCP source gives no universal imaging or tumour-marker schedule.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results NCCP 00464	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.
Written plan and contacts NCCP 00464	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

What each check is for

1

Treatment visits

21-day reference cycle

To control metastatic melanoma with chemotherapy, with benefit, blood counts and tolerability reviewed before each treatment.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00464

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.42

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Dacarbazine by IV infusion on day 1, repeated every 21 days for up to six cycles, or stopped earlier for progression or unacceptable toxicity.

Treatment timing: displayed days 1–21

Treatment or check	Days in this window	What happens
Treatment visit	Day 1	The NCCP dose is dacarbazine 850 mg/m² on day 1 in one litre of saline over one hour, with the infusion protected from light. The individual dose is prescribed using body size and clinical review. Allow extra time for blood results, anti-sickness medicine, preparation and any other fluids. Tell the nurse immediately about pain, burning, swelling or redness around the infusion.
Between-visit monitoring	Days 2–21	Follow your unit's fever threshold and urgent-contact instructions; call immediately if you develop fever, shaking chills or feel suddenly unwell, even without a high temperature. Also report unusual bleeding, persistent vomiting, inability to drink, reduced urine, jaundice or severe abdominal pain. Fever with abdominal pain and jaundice needs urgent assessment for rare severe liver injury. Tell the nurse immediately about infusion-site burning or swelling.

Schedule and review: Dacarbazine by IV infusion on day 1, repeated every 21 days for up to six cycles, or stopped earlier for progression or unacceptable toxicity.

Holds, missed treatment and phase changes: If treatment is delayed for low blood counts, illness or another problem, the team sets the new date and dose. Do not mark a planned but cancelled dose as received. The six-cycle limit is conditional on benefit and tolerability, not a requirement to give unsafe treatment.

> **Why these bands stay broad:** This is a reference schedule. Your prescribed dose, blood results, symptoms and the team's written plan determine whether treatment goes ahead. Band length does not show likelihood or

severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00464 v3: current regimen source ↗ · HPRA Dacarbazine 200 mg product information, March 2024 ↗

There are no guaranteed "safe days". The prescribed dose, treatment decision and local urgent-contact plan take priority. Call 112 or 999 for severe breathing difficulty, severe chest pain, collapse, a new seizure, sudden weakness or speech difficulty, or uncontrolled bleeding. Follow your oncology unit's written urgent-contact plan for other new or worsening symptoms; do not wait for the next appointment.

Complete timing and symptom notes

Day 1: Treatment day

The NCCP dose is dacarbazine 850 mg/m² on day 1 in one litre of saline over one hour, with the infusion protected from light. The individual dose is prescribed using body size and clinical review. Allow extra time for blood results, anti-sickness medicine, preparation and any other fluids. Tell the nurse immediately about pain, burning, swelling or redness around the infusion. Report symptoms and previous reactions before treatment.

Day 2–21: Between treatment visits

Follow your unit's fever threshold and urgent-contact instructions; call immediately if you develop fever, shaking chills or feel suddenly unwell, even without a high temperature. Also report unusual bleeding, persistent vomiting, inability to drink, reduced urine, jaundice or severe abdominal pain. Fever with abdominal pain and jaundice needs urgent assessment for rare severe liver injury. Tell the nurse immediately about infusion-site burning or swelling. Do not wait for the next appointment to report urgent symptoms.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking or receiving treatment

The NCCP dose is dacarbazine 850 mg/m² on day 1 in one litre of saline over one hour, with the infusion protected from light. The individual dose is prescribed using body size and clinical review. Allow extra time for blood results, anti-sickness medicine, preparation and any other fluids. Tell the nurse immediately about pain, burning, swelling or redness around the infusion.

STEP 2

Missed, vomited or held treatment

If treatment is delayed for low blood counts, illness or another problem, the team sets the new date and dose. Do not mark a planned but cancelled dose as received. The six-cycle limit is conditional on benefit and tolerability, not a requirement to give unsafe treatment.

STEP 3

Appointments and supply

Arrange blood tests, transport, medicine supply and interpreter or carer support. Check who reviews results and how you receive the decision before the next dose.

STEP 4

Paper copies and contacts

Keep the full handbook, three-page summary and wallet card together. The QR code is optional. Ask the team to replace every labelled example contact with your real oncology and out-of-hours details; ask for a new printed plan when treatment changes.

Sources: NCCP 00464 v3: current regimen source. Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Supportive medicines and symptom plan

Dacarbazine has high nausea-and-vomiting risk, so a planned preventive anti-sickness regimen is needed under the local protocol, including clear take-home instructions. The NCCP line that routine premedication is usually unnecessary does not remove this antiemetic requirement. Ask what to take regularly, what is rescue treatment and when to call if it fails.

2 Skin and daily care

Hair loss and sun sensitivity can occur, though they are not inevitable. Use gentle skin care and sun protection. Report redness, pain or swelling at a previous infusion site, or a spreading rash or signs of an allergic reaction.

3 Fertility, pregnancy and breastfeeding

The current HPRA product information advises effective contraception during treatment and for six months after the final dose for women, and during treatment and for three months afterwards for men. Discuss fertility preservation and, when relevant, genetic counselling before treatment. Do not breastfeed during treatment; ask the team about the appropriate interval afterwards.

Sources: nccp · dacarbazine-pi

TREATMENT REFERENCE · 01

Understanding your diagnosis and treatment goal

Use your own pathology, stage and agreed plan alongside this guide.

Your diagnosis and stage

Keep the pathology report, original melanoma site, stage and known sites of spread. This guide concerns advanced disease that cannot be removed completely by surgery or has spread; it does not describe a postoperative recurrence-prevention course.

Sources: nccn-patient

The tumour test

A BRAF V600 test helps identify whether BRAF-targeted medicines are an option. A tumour mutation does not by itself mean an inherited family condition. Keep the exact result and ask how it changes your choices.

Sources: nccn-patient

Scans and local treatments

Record the scans used to follow known disease. Surgery or radiotherapy may have a separate role for a particular site; ask for their timing and risks to be added to the overall plan.

Sources: nccn-patient

New changes still matter

Report a new lump, changing mole, persistent skin sore or unexplained symptom. A response at one site does not establish that every other symptom is harmless.

Sources: nccn-patient

TREATMENT REFERENCE · 02

Your medicines and overall plan

To control metastatic melanoma with chemotherapy, with benefit, blood counts and tolerability reviewed before each treatment.

The schedule

Dacarbazine by IV infusion on day 1, repeated every 21 days for up to six cycles, or stopped earlier for progression or unacceptable toxicity.

Sources: nccp · dacarbazine-pi

How treatment works

Dacarbazine is converted in the body into substances that damage DNA and make it harder for cancer cells to divide. It also affects some healthy cells, including blood-forming cells. This explains why nausea prevention and blood-count checks are central to the plan.

Sources: nccp · dacarbazine-pi

Important boundaries

This is single-agent dacarbazine at the NCCP 850 mg/m² dose. It is not an immune-checkpoint or BRAF-targeted regimen. Historical studies often used 1000 mg/m² and different treatment settings; their response or survival figures are not a precise forecast for this 850 mg/m² course.

Sources: nccp · dacarbazine-pi

Evidence and choices

The dacarbazine comparator response rate was 13.9%; this is not a validated efficacy estimate for NCCP dacarbazine 850 mg/m². These are group results from that study, not a prediction for you. Dacarbazine dose differs from NCCP 00464, and the population was BRAF wild type. The study primarily established benefit of nivolumab over a historical comparator. No exact survival or response prediction is assigned to the NCCP 850 mg/m² course.

Sources: checkmate066-context · nccp

TREATMENT REFERENCE · 03

Taking or receiving treatment safely

Keep the written prescription and actual treatment record together.

Administration

The NCCP dose is dacarbazine 850 mg/m² on day 1 in one litre of saline over one hour, with the infusion protected from light. The individual dose is prescribed using body size and clinical review. Allow extra time for blood results, anti-sickness medicine, preparation and any other fluids. Tell the nurse immediately about pain, burning, swelling or redness around the infusion.

Sources: nccp · dacarbazine-pi

Missed or held treatment

If treatment is delayed for low blood counts, illness or another problem, the team sets the new date and dose. Do not mark a planned but cancelled dose as received. The six-cycle limit is conditional on benefit and tolerability, not a requirement to give unsafe treatment.

Sources: nccp · dacarbazine-pi

Other medicines

Give the team a complete list of prescribed, over-the-counter and herbal medicines, supplements and allergies. Ask before adding a new medicine; treatment interactions and symptom-masking can matter.

Sources: nccp · dacarbazine-pi

Practical arrangements

Know where and when bloods are taken, how results reach the prescriber, and how to obtain the next supply or appointment. Ask for written instructions in a format you can use without a phone.

Sources: nccn-patient

TREATMENT REFERENCE · 04

Tests, results and treatment decisions

Safety tests and scans answer different questions.

Before treatment

Full blood count, kidney and liver function, medicines and prior treatment history. Discuss pregnancy prevention and fertility before the first dose. Record symptoms and the disease sites being followed on scans.

Sources: nccp · dacarbazine-pi

During treatment

Full blood count, kidney and liver function before every cycle. Blood-count suppression can be delayed: the product information describes the lowest counts often three to four weeks after a dose, so a 21-day appointment does not guarantee recovery. Extra blood tests are arranged when clinically needed.

Sources: nccp · dacarbazine-pi

What a result means

Ask which result is abnormal, whether the finding needs a repeat test and whether treatment is held, changed or stopped. Do not use a reference range or one normal result to decide independently that a dose is safe.

Sources: nccp · dacarbazine-pi

Response and follow-up

Keep reporting fever, bleeding, worsening fatigue or other symptoms after the last infusion while counts and organs recover. Response scans, later blood tests and the next treatment decision are individual; the NCCP source gives no universal imaging or tumour-marker schedule.

Sources: nccp · dacarbazine-pi

TREATMENT REFERENCE · 05

What to expect between visits

A quiet treatment day does not rule out later problems.

Act on new symptoms

Follow your unit’s fever threshold and urgent-contact instructions; call immediately if you develop fever, shaking chills or feel suddenly unwell, even without a high temperature. Also report unusual bleeding, persistent vomiting, inability to drink, reduced urine, jaundice or severe abdominal pain. Fever with abdominal pain and jaundice needs urgent assessment for rare severe liver injury. Tell the nurse immediately about infusion-site burning or swelling.

Sources: nccp · dacarbazine-pi

Supportive medicines

Dacarbazine has high nausea-and-vomiting risk, so a planned preventive anti-sickness regimen is needed under the local protocol, including clear take-home instructions. The NCCP line that routine premedication is usually unnecessary does not remove this antiemetic requirement. Ask what to take regularly, what is rescue treatment and when to call if it fails.

Sources: nccp · dacarbazine-pi

A useful daily record

Record symptoms, temperature when unwell, bowel changes, fluid intake when reduced, missed doses and any medicines used for relief. Note how symptoms affect eating, walking, sleep and normal activities; this helps the team judge severity.

Sources: nccp · dacarbazine-pi

When life is affected

Tell the team about fatigue, pain, appetite, sleep, mood and ability to work or care for others. Practical and supportive care can accompany anticancer treatment; you do not have to wait until treatment ends.

Sources: nccn-patient

TREATMENT REFERENCE · 06

Skin, body changes and daily care

Use the detailed body map to locate symptoms, not to diagnose them.

Skin and sun

Hair loss and sun sensitivity can occur, though they are not inevitable. Use gentle skin care and sun protection. Report redness, pain or swelling at a previous infusion site, or a spreading rash or signs of an allergic reaction.

Sources: nccp · dacarbazine-pi

The body map

The organ anchors show possible locations and warning symptoms. They do not show how common each effect is, and a symptom may have another cause. New symptoms should be assessed in their clinical context.

Sources: nccp · dacarbazine-pi

Food, fluids and activity

Keep meals and fluids manageable and follow any individual fluid or diet restrictions. Gentle activity and rest can be adjusted to how you feel. Ask for dietetic or rehabilitation help if intake, weight, strength or mobility is falling.

Sources: nccn-patient

Keep checking the plan

A new medicine, fever, hospital admission, procedure or radiotherapy may change treatment timing. Tell the oncology team and take the wallet card or medicine list to other appointments.

Sources: nccp · dacarbazine-pi

TREATMENT REFERENCE · 07

Fertility, pregnancy and breastfeeding

Discuss these issues before the first dose whenever possible.

Contraception and pregnancy

The current HPRA product information advises effective contraception during treatment and for six months after the final dose for women, and during treatment and for three months afterwards for men. Discuss fertility preservation and, when relevant, genetic counselling before treatment. Do not breastfeed during treatment; ask the team about the appropriate interval afterwards.

Sources: nccp · dacarbazine-pi

Fertility preservation

Ask whether treatment or the cancer may affect fertility and whether fertility preservation is appropriate before treatment. A change in periods or sexual function does not reliably establish infertility.

Sources: nccp · dacarbazine-pi

Written dates and methods

Record the agreed method, last treatment date and how long precautions continue. Different medicines have different intervals, so ask for clarification after a switch or combination change.

Sources: nccp · dacarbazine-pi

Sexual health

Tell the team about sexual symptoms, relationship concerns or practical difficulties with the agreed precautions. Ask for a private discussion and specialist support when needed.

Sources: nccn-patient

TREATMENT REFERENCE · 08

Decisions, support and useful records

Your priorities belong in the treatment discussion.

Benefits and alternatives

Ask what the treatment aims to achieve, what alternatives are suitable, how likely important benefits are in your situation and how treatment burden or uncertainty affects that choice. A trial average cannot promise an individual outcome.

Sources: checkmate066-context · nccp

Support alongside treatment

Ask about symptom control, psychological support, finances, transport, work and help at home. Palliative or supportive care can be used alongside anticancer treatment to improve comfort and function.

Sources: nccn-patient

A person you trust

If helpful, bring a family member or supporter and agree what information may be shared. Ask for an interpreter or accessible information when needed; use teach-back to check the schedule and urgent plan.

Sources: nccn-patient

Your document set

Keep consent information, the three-page summary, complete handbook, treatment card, test plan and real contact details together. The QR opens more information but essential instructions remain available in print.

Sources: nccn-patient

TREATMENT REFERENCE · 09

After a treatment hold, change or completion

The next step should be written down, not assumed.

Follow-up and late effects

Keep reporting fever, bleeding, worsening fatigue or other symptoms after the last infusion while counts and organs recover. Response scans, later blood tests and the next treatment decision are individual; the NCCP source gives no universal imaging or tumour-marker schedule.

Sources: nccp · dacarbazine-pi

Restarting or changing treatment

If treatment is delayed for low blood counts, illness or another problem, the team sets the new date and dose. Do not mark a planned but cancelled dose as received. The six-cycle limit is conditional on benefit and tolerability, not a requirement to give unsafe treatment.

Sources: nccp · dacarbazine-pi

A treatment summary

Record medicines and doses received, important side effects, admissions, last-dose date, ongoing symptom treatment and the clinician responsible for follow-up. Ask for an updated paper copy for your GP and other treating teams.

Sources: nccn-patient

Your next review

Confirm the next blood test, scan and review, who explains the result, and which symptoms should trigger contact sooner. Keep actual daytime and out-of-hours numbers accessible after treatment stops.

Sources: nccn-patient

What the treatment evidence can and cannot tell you

Evidence needs to match the medicine, dose combination, disease stage and previous treatment.

The dacarbazine comparator response rate was 13.9%; this is not a validated efficacy estimate for NCCP dacarbazine 850 mg/m². These are group results from that study, not a prediction for you.

Sources: checkmate066-context · nccp

PREPARE FOR THE CONVERSATION

Questions worth asking

What is the treatment schedule?

Dacarbazine by IV infusion on day 1, repeated every 21 days for up to six cycles, or stopped earlier for progression or unacceptable toxicity.

What if a dose is missed or held?

If treatment is delayed for low blood counts, illness or another problem, the team sets the new date and dose. Do not mark a planned but cancelled dose as received. The six-cycle limit is conditional on benefit and tolerability, not a requirement to give unsafe treatment.

Which symptoms need early contact?

Follow your unit's fever threshold and urgent-contact instructions; call immediately if you develop fever, shaking chills or feel suddenly unwell, even without a high temperature. Also report unusual bleeding, persistent vomiting, inability to drink, reduced urine, jaundice or severe abdominal pain. Fever with abdominal pain and jaundice needs urgent assessment for rare severe liver injury. Tell the nurse immediately about infusion-site burning or swelling.

Which tests are needed?

Full blood count, kidney and liver function, medicines and prior treatment history. Discuss pregnancy prevention and fertility before the first dose. Record symptoms and the disease sites being followed on scans. Full blood count, kidney and liver function before every cycle. Blood-count suppression can be delayed: the product information describes the lowest counts often three to four weeks after a dose, so a 21-day appointment does not guarantee recovery. Extra blood tests are arranged when clinically needed. Keep reporting fever, bleeding, worsening fatigue or other symptoms after the last infusion while counts and organs recover. Response scans, later blood tests and the next treatment decision are individual; the NCCP source gives no universal imaging or tumour-marker schedule.

Can I use this without a phone?

Yes. The complete handbook, three-page summary and cut-out wallet card are printable. Ask the team to enter actual contact numbers and provide an updated paper plan when anything changes.

> **Try explaining the plan back:** Can you explain your treatment setting, medicine schedule, next tests, the symptoms that need urgent advice and the number you would call?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00464 v3. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00464 v3: current regimen source
- HPRA Dacarbazine 200 mg product information, March 2024
- NCCN Guidelines for Patients: Melanoma, 2026 — supplied education reference
- Robert et al. CheckMate 066, NEJM 2015 — historical context only

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-06.

This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://assets.hse.ie/media/documents/464_V3_Dacarbazine.pdf

https://assets.hpra.ie/products/Human/19327/Licence_PA0623-003-002_04032024123758.pdf

https://www.nccn.org/patients/guidelines/content/PDF/melanoma-patient.pdf

https://pubmed.ncbi.nlm.nih.gov/25399552/

BASELINE → TREATMENT → FOLLOW-UP

Your testing and review plan

A reference cycle is not a personalised calendar. Tests listed as symptom-triggered or individual are not presented as mandatory tests at every visit. No universal imaging or tumour-marker interval is invented.

Check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms	Full blood count, kidney and liver function, medicines and prior treatment history. Discuss pregnancy prevention and fertility before the first dose. Record symptoms and the disease sites being followed on scans.	Full blood count, kidney and liver function before every cycle. Blood-count suppression can be delayed: the product information describes the lowest counts often three to four weeks after a dose, so a 21-day appointment does not guarantee recovery. Extra blood tests are arranged when clinically needed.	Follow your unit's fever threshold and urgent-contact instructions; call immediately if you develop fever, shaking chills or feel suddenly unwell, even without a high temperature. Also report unusual bleeding, persistent vomiting, inability to drink, reduced urine, jaundice or severe abdominal pain. Fever with abdominal pain and jaundice needs urgent assessment for rare severe liver injury. Tell the nurse immediately about infusion-site burning or swelling.	Keep reporting fever, bleeding, worsening fatigue or other symptoms after the last infusion while counts and organs recover. Response scans, later blood tests and the next treatment decision are individual; the NCCP source gives no universal imaging or tumour-marker schedule.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.
Written plan and contacts	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00464 v3: current regimen source · HPRA Dacarbazine 200 mg product information, March 2024

THE WHOLE TREATMENT JOURNEY

Dacarbazine — metastatic melanoma: treatment and checks

Advanced melanoma

Dacarbazine by IV infusion on day 1, repeated every 21 days for up to six cycles, or stopped earlier for progression or unacceptable toxicity.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms NCCP 00464	Full blood count, kidney and liver function, medicines and prior treatment history. Discuss pregnancy prevention and fertility before the first dose. Record symptoms and the disease sites being followed on scans.	Full blood count, kidney and liver function before every cycle. Blood-count suppression can be delayed: the product information describes the lowest counts often three to four weeks after a dose, so a 21-day appointment does not guarantee recovery. Extra blood tests are arranged when clinically needed.	Follow your unit's fever threshold and urgent-contact instructions; call immediately if you develop fever, shaking chills or feel suddenly unwell, even without a high temperature. Also report unusual bleeding, persistent vomiting, inability to drink, reduced urine, jaundice or severe abdominal pain. Fever with abdominal pain and jaundice needs urgent assessment for rare severe liver injury. Tell the nurse immediately about infusion-site burning or swelling.	Keep reporting fever, bleeding, worsening fatigue or other symptoms after the last infusion while counts and organs recover. Response scans, later blood tests and the next treatment decision are individual; the NCCP source gives no universal imaging or tumour-marker schedule.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results NCCP 00464	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.
Written plan and contacts NCCP 00464	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

What each check is for

1

Treatment visits

21-day reference cycle

To control metastatic melanoma with chemotherapy, with benefit, blood counts and tolerability reviewed before each treatment.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00464

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.42

Dacarbazine — metastatic melanoma

> **The treatment this guide covers** This is single-agent dacarbazine at the NCCP 850 mg/m² dose. It is not an immune-checkpoint or BRAF-targeted regimen. Historical studies often used 1000 mg/m² and different treatment settings; their response or survival figures are not a precise forecast for this 850 mg/m² course.
>
> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Separate scripts for each phase

Choose the active phase in the clinician workflow. Each script has its own medicines, timing and review checks.

- **Dacarbazine — metastatic melanoma · support plan** — High per NCCP 00464 v3; preventive antiemetic regimen required under local policy
  Dacarbazine

https://sactcheck.com/patient/00464/supportive-care.html

Dacarbazine — metastatic melanoma

The treatment this guide covers This is single-agent dacarbazine at the NCCP 850 mg/m² dose. It is not an immune-checkpoint or BRAF-targeted regimen. Historical studies often used 1000 mg/m² and different treatment settings; their response or survival figures are not a precise forecast for this 850 mg/m² course.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Dacarbazine — metastatic melanoma · support plan

Active medicines: Dacarbazine

Emetogenic risk: High per NCCP 00464 v3; preventive antiemetic regimen required under local policy

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1 Current NCIS V8 non-AC high-risk row; review interactions.
Ondansetron	16 mg orally	Day 1 Review QT risk, electrolytes, constipation and hepatic dosing.
Dexamethasone	12 mg orally	Day 1 Reconcile the complete steroid plan and aprepitant interaction.
Olanzapine	5 mg orally	Days 1–4 Review sedation, falls, glucose and interactions.
Aprepitant	80 mg orally once daily	Days 2–3 Check the take-home schedule.
Dexamethasone	8 mg orally once daily	Days 2–4 Non-AC high-risk schedule; do not substitute the AC row.
Metoclopramide	10 mg orally up to three times daily as needed	Breakthrough / subsequent days; duration to be prescribed Caution with olanzapine because of additive extrapyramidal effects; review renal function, contraindications and alternative rescue options.

Additional phase-specific support

- Use an individual approved high-risk antiemetic prescription, with prevention and breakthrough instructions. The source-based medicines below are a reviewable reference, not an automatic prescription; reconcile interactions, steroid exposure, sedation, QT risk and rescue alternatives.
- Dacarbazine has high nausea-and-vomiting risk, so a planned preventive anti-sickness regimen is needed under the local protocol, including clear take-home instructions. The NCCP line that routine premedication is usually unnecessary does not remove this antiemetic requirement. Ask what to take regularly, what is rescue treatment and when to call if it fails.
- Full blood count, kidney and liver function before every cycle. Blood-count suppression can be delayed: the product information describes the lowest counts often three to four weeks after a dose, so a 21-day appointment does not guarantee recovery. Extra blood tests are arranged when clinically needed.
- Follow your unit's fever threshold and urgent-contact instructions; call immediately if you develop fever, shaking chills or feel suddenly unwell, even without a high temperature. Also report unusual bleeding, persistent vomiting, inability to drink, reduced urine, jaundice or severe abdominal pain. Fever with abdominal pain and jaundice needs urgent assessment for rare severe liver injury. Tell the nurse immediately about infusion-site burning or swelling.
- The current HPRA product information advises effective contraception during treatment and for six months after the final dose for women, and during treatment and for three months afterwards for men. Discuss fertility preservation and, when relevant, genetic counselling before treatment. Do not breastfeed during treatment; ask the team about the appropriate interval afterwards.
- This is single-agent dacarbazine at the NCCP 850 mg/m² dose. It is not an immune-checkpoint or BRAF-targeted regimen. Historical studies often used 1000 mg/m² and different treatment settings; their response or survival figures are not a precise forecast for this 850 mg/m² course.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00464 v3 · p4 · NCCP NCIS medical-oncology antiemetics V8, Table 1 p4, high risk excluding AC

Sources checked 2026-11-06 · SACTCheck v0.84.42 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Dacarbazine — metastatic melanoma

NCCP 00464 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking or receiving treatment

The NCCP dose is dacarbazine 850 mg/m² on day 1 in one litre of saline over one hour, with the infusion protected from light. The individual dose is prescribed using body size and clinical review. Allow extra time for blood results, anti-sickness medicine, preparation and any other fluids. Tell the nurse immediately about pain, burning, swelling or redness around the infusion.

STEP 2

Missed, vomited or held treatment

If treatment is delayed for low blood counts, illness or another problem, the team sets the new date and dose. Do not mark a planned but cancelled dose as received. The six-cycle limit is conditional on benefit and tolerability, not a requirement to give unsafe treatment.

STEP 3

Appointments and supply

Arrange blood tests, transport, medicine supply and interpreter or carer support. Check who reviews results and how you receive the decision before the next dose.

STEP 4

Paper copies and contacts

Keep the full handbook, three-page summary and wallet card together. The QR code is optional. Ask the team to replace every labelled example contact with your real oncology and out-of-hours details; ask for a new printed plan when treatment changes.

Sources: NCCP 00464 v3: current regimen source. Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending.

Dacarbazine — metastatic melanoma

A record of what you experience

One cycle per page. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal CTCAE grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Dacarbazine by IV infusion on day 1, repeated every 21 days for up to six cycles, or stopped earlier for progression or unacceptable toxicity.

These pages are reusable recording sheets; the prescribed course length is stated above. Ask for additional sheets as needed.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Your own hospital / contact instructions:

https://sactcheck.com/patient/00464/

Full patient information, sources and visual timeline. No identifiers are encoded.

Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending

My diagnosis, plan and medicines

Complete these pages with your team. Leave unknown or inapplicable items blank. This record supports the consent discussion and your written prescription.

Diagnosis and overall plan

Original melanoma site / pathology date

Stage / Breslow thickness / ulceration

Lymph-node or other disease sites

BRAF result / date / other relevant tumour tests

Advanced or adjuvant intent / earlier surgery or RT

Medicines and safety

Anticancer medicines / actual dose / route / start date

Treatment received, missed or held / restart instruction

All other medicines / allergies / important previous reactions

Pregnancy-prevention or immune-alert plan / continuing precautions

Oncology / out-of-hours / pharmacy contacts

NCCN Guidelines for Patients: Melanoma, 2026 - supplied education reference; original fields adapted to the Irish NCCP regimen context. No NCCN form or artwork reproduced.. Your Irish treating team confirms the diagnosis, prescription and follow-up plan.

My checks, symptoms and support

Record what each test is checking and when the result will be discussed. Your team explains which examinations or scans are needed for your diagnosis and treatment.

Test and purpose	Date / booking contact	Result discussion	Agreed next step

Record bloods, skin/lesion examinations, scans and specialist checks separately. Tumour-marker and scan intervals are individually selected; do not infer them from the cycle calendar.

Symptoms, skin and support

New symptoms / onset / change / action taken

Lesion or wound / pain / eating / weight / daily function

Skin self-check plan / optional dated photograph record kept privately

Sun protection / practical, emotional or family support needs

Questions, response review and the outcomes that matter to me

My urgent contact plan / what my carer should do

NCCN Guidelines for Patients: Melanoma, 2026 - supplied education reference; original fields adapted to the Irish NCCP regimen context. No NCCN form or artwork reproduced.. Your Irish treating team confirms the diagnosis, prescription and follow-up plan.

Dacarbazine — metastatic melanoma

Cycle number: _____ Start date: _____ Next review: _____

Record the dose actually taken or administered, symptoms, tests and agreed next step. A planned or held visit is not a completed treatment dose.

Dacarbazine — metastatic melanoma · copy this sheet for another cycle of the same phase.

Dacarbazine · Cytotoxic chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999
Fever / chills	☐ No fever or chills	☐ New chills / feeling unwell: call	☐ Fever: use the written urgent plan now	☐ Fever with marked weakness: urgent assessment	☐ Collapse / confusion / breathing difficulty: 112/999
Bleeding / bruising	☐ None	☐ New small bruise / brief bleed: report	☐ Repeated bleeding: call today	☐ Bleeding that will not stop: urgent help	☐ Heavy bleeding / collapse: 112/999
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999
Infusion-site symptoms	☐ None	☐ New burning or pain during infusion: tell the nurse immediately	☐ Swelling, redness or leakage: tell the nurse immediately	☐ Increasing pain, blistering or spreading redness: urgent review	☐ Severe sudden illness or collapse: emergency help

Dates / doses missed or changed / symptoms affecting daily life / questions:

> Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Keep your treatment details to hand

Advanced melanoma · NCCP 00464 v3 · Document revised 6 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Dacarbazine

Dacarbazine by IV infusion on day 1, repeated every 21 days for up to six cycles, or stopped earlier for progression or unacceptable toxicity.

Phase: _____ Planned cycles: 6

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Team stamps after each completed cycle.

C1	C2	C3
Date / initials	Date / initials	Date / initials
C4	C5	C6
Date / initials	Date / initials	Date / initials

My treatment information

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.42 · NCCP 00464 v3 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.